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/ **Practice Exam - 1**

Correct Answer

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Practice Exam - 1

Question 11 of 117



A 48-year-old woman presents with worsening dyspnea on exertion for the last 4 months. She reports mild intermittent leg edema, but denies chest pain, palpitations or syncope. Physical exam reveals JVP of 9cmH₂O at 45 degrees and 1+ bilateral lower leg edema. The rest of the physical exam is normal. The patient states that she takes Warfarin since an abnormal V/Q scan a year ago. NT-pro-BNP is 2690 ng/L. She covers 240 meters during a 6-minute walk test.

Echocardiogram shows mild RV enlargement, mildly reduced RV systolic function, RVSP of 40 mmHg, and trace tricuspid valve regurgitation. V/Q scan is unchanged. Chest CT angiography shows multiple bilateral segmental pulmonary emboli. RHC reveals RAP 9mmHg, severe pulmonary hypertension 62/28/45mmHg, cardiac index of 2.0 L/min/m² and PCWP of 12mmHg.

Which therapeutic approach would have the most durable effect in this patient?

- A. Change Warfarin to rivaroxaban
-  B. Recommend a catheter directed thrombolytic therapy
- C. Initiate riociguat therapy
- D. Refer for balloon pulmonary angioplasty
-  E. Refer to surgical pulmonary thromboendarterectomy

Commentary

References

Testing Point: Exam Takers should know that the optimal treatment for operable chronic, multi-segmental CTEPH is pulmonary thromboendarterectomy (PTE).

Answer: E. Refer for surgical pulmonary thromboendarterectomy

Rationale: This patient has operable Chronic Thromboembolic Pulmonary Hypertension (WHO Group 4 pulmonary hypertension), demonstrated by persistent mismatched perfusion defects, chronic thromboembolic disease on CTA, and precapillary pulmonary hypertension on right heart catheterization. Pulmonary thromboendarterectomy is the treatment with the greatest potential for durable hemodynamic improvement and long-term survival and may be curative in eligible patients.

Answer C: Riociguat is approved for patients with inoperable CTEPH or persistent/recurrent pulmonary hypertension after surgery, but it provides less durable hemodynamic improvement than surgical pulmonary thromboendarterectomy in patients with operable disease.

Answer A: Switching anticoagulants is unlikely to meaningfully alter disease progression.

Answer B: Catheter-directed thrombolysis is generally used for acute pulmonary embolism and is less effective for chronic organized thrombus.

Answer D: Balloon pulmonary angioplasty can improve hemodynamics and symptoms in patients with inoperable CTEPH or residual pulmonary hypertension after surgery, but pulmonary thromboendarterectomy remains the preferred therapy in operable disease.

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